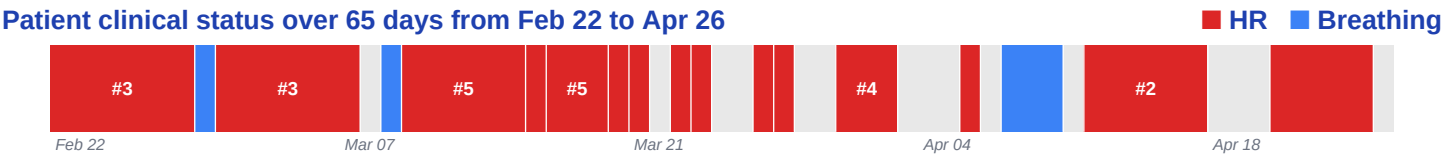


CLINICAL INTELLIGENCE TREND REPORT

YELLOW: Closer Observation Suggested

Patient ID: PHolst **Period:** February 22 to April 26, 2024 **Report Date:** April 27, 2024 **Coverage:** 1432/1537h (78.9%)

Patient's monitoring period was less than 90 days; this report covers all available data. Decision-support summary derived from longitudinal vital sign trends; interpret in clinical context.



Episodic Events	Total Hours	Highest Burden	Episodes/day	Coverage
86	124h	Very High Heart Rate	1	93%

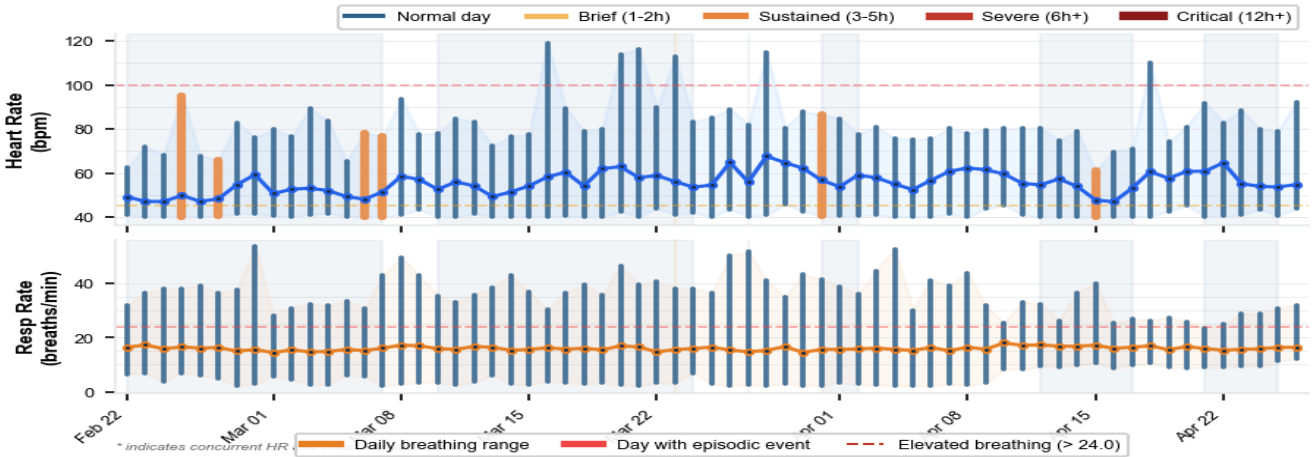
Major Findings: Sustained very high HR (avg 56, peak 119)

#	Episode	Min/Max	Total Hours	Longest Continuous	Episodes/d ay	Average	Date
1	Very High Heart Rate	113 bpm	1h	1h	1	56 bpm	Mar 23
2	Low Heart Rate	40 bpm	21h	4h	2	52 bpm	Apr 12 to Apr 17
3	Low Heart Rate	40 bpm	56h	3h	3	51 bpm	Feb 22 to Mar 07
4	Low Heart Rate	40 bpm	9h	3h	2	56 bpm	Mar 31 to Apr 02
5	Low Heart Rate	40 bpm	20h	2h	1	56 bpm	Mar 10 to Mar 24
6	Low Heart Rate	40 bpm	5h	2h	4	56 bpm	Mar 27

+ 2 additional condition(s) across High Breathing, Low Heart Rate; details in monitoring data.

The P5 to P95 heart rate spread was 26 bpm (44 to 70), indicating significant cardiac variability. RR values outside physiologic range (6–60 breaths/min).

- Clinical Pattern Observations:**
- 1. **High variability:** HR spread 26 bpm (43 to 70).
 - 2. **Clustered pattern:** Episodic events on 6 of 65 days (9%).
 - 3. **Nocturnal pattern:** 5 of 6 eligible episodes during evening or night hours.



Red bars indicate days with detected episodic events.

■ HR episode ■ RR episode ■ Recorded, no episode ■ No data

Clinical Alerting Thresholds

Very Low HR < 40 bpm	Low HR < 45 bpm	Elevated HR > 95 bpm	High HR > 100 bpm
Very High HR > 110 bpm	Elevated Breathing > 24 bpm	High Breathing > 30 bpm	Very High Breathing > 40 bpm

Episodic event: threshold exceeded continuously for ≥ 1 hour. Episodes separated by ≤ 1 hour of normal vitals are counted as the same event. Brief threshold crossings that do not sustain are not flagged.